

Body Signal Index

FOUNDATION REFERENCE

Somatic literacy reference for reading body signals with precision

Asset ID: DC-R04-BSI | Revision v0.1

The Sovereign Bureau | A. Solenne Institute | A division of Granite Field Holdings Ltd. Co.

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Body Signal Index is an educational and self-development resource within the Distinct Character ecosystem. It is designed for somatic literacy, behavioral governance, and structured self-observation.

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If you are experiencing acute distress, medical symptoms, self-harm thoughts, dissociation, panic, severe depression, eating-disorder symptoms, or any condition requiring clinical support, stop using the resource and contact an appropriate licensed professional or emergency service in your location.

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BODY SIGNAL INDEX

Distinct Character | Biological Infrastructure Companion

A somatic literacy reference for reading the body with precision. Not a assessment-oriented instrument. Use in conjunction with the Somatic Architecture Cycle.

Signal Reference Table

TENSION AND BREATH		
Jaw tension or clenching	Suppressed speech, held boundary, or chronic low-level stress.	<i>Jaw release; name what is being held back.</i>
Shallow or held breath	Threat response active; window of tolerance narrowing.	<i>Elongated exhale; orienting breath.</i>
Chest tightness	Anxiety, unprocessed emotion, or grief.	<i>Breath work; name the emotion precisely.</i>
Neck or shoulder holding	Responsibility load, over-efforting, or performance pressure.	<i>Physical release; assess overfunctioning pattern.</i>
ENERGY AND CAPACITY		
Heavy limbs or fatigue	Depletion, recovery deficit, or parasympathetic shutdown.	<i>Rest without guilt; assess sleep and load.</i>
Flat affect or heaviness	Approaching or inside depletion; possible depression signal.	<i>Rest; medical check-in if persistent.</i>
Irritability without clear cause	Elevated cortisol load, depleted capacity, or unmet need.	<i>Do not make decisions; regulate first.</i>
ACTIVATION AND DYSREGULATION		
Restlessness	Sympathetic activation, unmet need, or suppressed movement.	<i>Movement discharge; then stillness.</i>
Buzzing or hypervigilance	Sympathetic overdrive; threat perception elevated.	<i>Sensory reduction; slow movement; stillness.</i>
SHUTDOWN AND DISSOCIATION		
Numbness or disconnection	Dissociation, overwhelm past threshold, or freeze response.	<i>Grounding; gentle orienting; safe co-regulation.</i>
SOMATIC AND DIGESTIVE		
Digestive disruption	Elevated cortisol, nervous system activation, or suppressed stress.	<i>Regulate before eating; assess allostatic load.</i>

Usage Protocol: The Somatic Architecture Cycle

When a signal is identified, move through these steps in sequence. The cycle does not require a single sitting. Notice and Name happen in seconds. Adjust may take days.

02	NOTICE: Identify the signal before labeling it. Precision precedes interpretation.
03	NAME: Match the signal to this index. Use the framework language, not casual shorthand.
04	REGULATE: Apply the indicated first response. Do not skip to analysis while the body is still activated.
05	RECOVER: Allow full restoration before re-engaging with high-demand work or decisions.
	ADJUST: Use the signal as data. Make a structural change to load, environment, or schedule if the signal repeats.

Signal Interpretation Notes

- Signals that appear in clusters indicate broader regulation load, not isolated symptoms. If three or more signals from the Activation or Tension categories present simultaneously, treat this as a possible Stage 2 or Stage 3 depletion indicator.
- Recurring signals that return within 24 to 48 hours after regulation indicate a structural problem in load, environment, or recovery architecture, not a personal regulation failure. Refer to the Depletion and Recovery Map.
- Flat affect, numbness, or persistent heaviness lasting more than three days should be assessed by a licensed professional. This index does not replace clinical support.
- For those managing chronic illness: signals may not follow the standard intensity-to-cause pattern. Use this index as a starting reference and calibrate against your known baseline, not population norms.